

FENOLIP-M CAPSULE 200MG

DESCRIPTION:

Opaque orange/ opaque orange colored size '1' hard gelatin capsules.

COMPOSITION:

Each capsule contains:

Fenofibrate micronised 200mg

ACTION AND PHARMACOLOGY:

Hypolipidemic: Studies showed that Fenofibrate can lower blood cholesterol by 20- 30%, LDL and VLDL- Cholesterol by 20-35% and triglycerides by 40-55%. The levels of HDL-cholesterol are frequently increased. The overall effect is a decrease of LDL and VLDL to HDL ratio. This decreased ratio improves plasma cholesterol proportions, which epidemiological studies have correlated with a decrease in atherogenic risk. Fenofibrate has a uricosonic effect and is therefore especially beneficial for type IV hyperlipidemic patients, who very often have increased plasma uric acid level.

Fenofibrate is readily absorbed from the gastro-intestinal tract when taken with food; absorption is substantially reduced if it is administered after an overnight fast. It is rapidly hydrolysed to its active metabolite fenofibric acid which is extensively bound to plasma albumin. The plasma half-life of elimination of fenofibric acid is approximately 20 hrs.

Fenofibric acid is excreted predominantly in the urine, mainly as the glucuronide conjugate, but also as a reduced form of fenofibric acid and its glucuronide. Kinetic studies after multiple dosing show the absence of accumulation of the product. The micronized design employs a more efficient delivery system by improving pharmaceutical quality and reduce variability of absorption. Hence, this improves the reproducibility of the pharmacological & therapeutic effect. Fenofibric acid is not eliminated during dialysis.

INDICATIONS:

Treatment hypercholesterolaemia (abnormally elevated levels of cholesterol in the blood) and /or hypertriglyceridaemia (abnormally elevated levels of triglycerides, fats in the blood), in patients in whom dietary measures alone have failed to produce anadequate response. It is therefore indicated in appropriate cases of hyperlipidaemia (Fredrickson classification type Ha, IIb, III, IV and V).

CONTRAINDICATIONS:

Hypersensitivity to fenofibrate, severe liver dysfunction, existing gallbladder disease and severe renal disorders. Also contraindicated in children.

Known photoallergy or phototoxic reactions during treatment with fibrates or ketoprofen.

SIDE EFFECTS:

Adverse reactions observed during treatment are rare. It is generally minor, transient and do not interfere with treatment. Most commonly reported are mild gastro-intestinal disturbances, skin reactions, headache, fatigue, and vertigo. Sexual asthenia and muscle cramps are less frequently reported. Rarely: hepatitis, rhabdomyolysis, gallstones, photosensitivity.

WARNING AND PRECAUTIONS:

Dose reduction should be considered in elderly patients and those with impaired renal and hepatic functions. If satisfactory reduction of serum concentration of lipids is not obtained after 3 to 6 months of treatment, other therapeutic means (complementary or different) must be envisaged. Systemic verification of transaminases every 3 months, over the first 12 months of treatment should be done. Patients who develop signs of muscle toxicity should be monitored closely and CPK levels checked. Treatment with fenofibrate should be stopped if myopathy is suspected or if CPK rises to greater than or equal 10 times the upper limit of normal.

DRUG INTERACTIONS:

Oral anticoagulants: Fenofibrate interacts with oral anticoagulants and enhances the haemorrhagic risk through displacement of their binding with the plasma proteins. Therefore the dose of anticoagulants should be reduced by about one third at the commencement of treatment and then gradually adjusted if necessary.

HMG-CoA reductase inhibitors: Concurrent use of lovastatin (or other HMG-CoA reductase inhibitors) may cause severe myositis and myoglobinuria.

Cyclosporine: Some severe cases of reversible renal function impairment have been reported during concomitant administration of fenofibrate and cyclosporine. The renal function of these patients must therefore be closely monitored and the treatment with fenofibrate stopped in the case of severe alteration of laboratory parameters.

Others: Possible interaction with oral hypoglycaemic agents should be considered.

PREGNANCY & LACTATION:

Fenofibrate has not shown any teratogenic effects. However, there is no conclusion to the absence of congenital malformations in human. There is no information of the passage of fenofibrate into the mother's milk. Therefore, it is recommended not to use in pregnant & lactating women.

DOSAGE AND ADMINISTRATION:

One capsule daily to be taken during one of the main meals.

ROUTE OF ADMINISTRATION:

Oral administration

OVERDOSAGE AND TREATMENT:

No reports of ill effects from overdose have been reported. There are no specific antidotes and treatment of acute overdose should be symptomatic. Gastric lavage and appropriate supportive care may be instituted if necessary.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES:

No negligible influence on the ability to drive and use machines.

PRESENTATION:

Blister strips of 10 capsules per strip; boxes of 3 strips and 10 strips.

STORAGE:

Store in a dry place below 30°C.

Keep container tightly closed.

Protect from light.

Keep out of reach of children.

MANUFACTURED BY:

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